

Public Health Emergency Operations Center
"COUSP DRC"

MVE B-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°074/MVE B_27/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (48)	- Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba - Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gethy, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka and Musienene. - South Kivu (1/34): Miti-Murhesa - Tshopo (3/23): Lubunga, Makiso-Kisangani and Mangobo
Reporting date	July 27, 2026
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KEY INDICATORS — CUMULATIVE TO JULY 27, 2026

CONFIRMED CASES — 5 PROVINCES 3,360 +98 new cases (Ituri 87 · N-Kivu 11)	CONFIRMED DEATHS · LEATHITIS 1,487 (44.3%) +50 deaths (Ituri 31 · North Kivu 19)	SUSPECTED CASES OF THE DAY 321 Ituri 190 · N-Kivu 105 · Haut-Uélé 25 · Tshopo 1
PATIENTS IN ISOLATION 733 Ituri 544 · North Kivu 189	CURED — CUMULATIVE 597 +14 recovered (Ituri 13 – North Kivu 1)	NATIONAL CONTACT TRACKING 78.9% 13,882/ 17,588 views · Ituri 77.6% - Top-Uele 60.0% - N-Kivu 82.6% - Tshopo 100.0%

* Harmonisation of number of deaths by health zone in the province of North Kivu (+ 11 deaths).

1. Epidemiological Highlights

- Epidemiological situation:** No new health zones affected. The total remains at 48 affected health zones. 140 in the five provinces, of which 47 remain in active transmission.
- Case trends:** In the last 24 hours, 98 new confirmed cases, 50 confirmed deaths and 14 recoveries were recorded:
 - o Ituri: 87 new confirmed cases were notified, mainly in Bunia (28), Mongbwalu (22), Rwampara (17), Kilo (5), Lita (4), Mandima (3), Nizi (3), Nia-Nia (2), Komanda (1), Mahagi (1) and Tchomia (1), 31 deaths among which 51.6% were community deaths.
 - North **Kivu**: 11 new confirmed cases were reported (Butembo 2, Katwa 5, Beni 2 and Musienene 2) as well as 19 deaths (after harmonization with health zones, see Table 2), including 5 community deaths. The provincial total reaches 325 confirmed cases and 223 deaths, representing a case fatality rate of 68.6%.

- o **Haut-Uélé** : No new confirmed cases or deaths have been reported. The provincial total remains at 39 confirmed cases and 21 deaths, representing a case fatality rate of 53.8%, spread across 5 out of 13 health zones in the province.
- o **South Kivu and Tshopo** : No new confirmed cases or deaths have been reported. The cumulative totals remain at 5 cases, 4 deaths, and 1 recovery for Tshopo, and 3 confirmed cases, including 1 death and 2 recoveries, for South Kivu. The Miti-Murhesa health zone has a total of 62 days while the Tshopo province has 13 days without a new confirmed case.

- **Analysis:** Ituri remains the epicenter of the epidemic, accounting for 88.9% of cumulative cases and 88.8% of new daily confirmations. Contact tracing performance remains insufficient and requires strengthened surveillance activities.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it

The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 48 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.

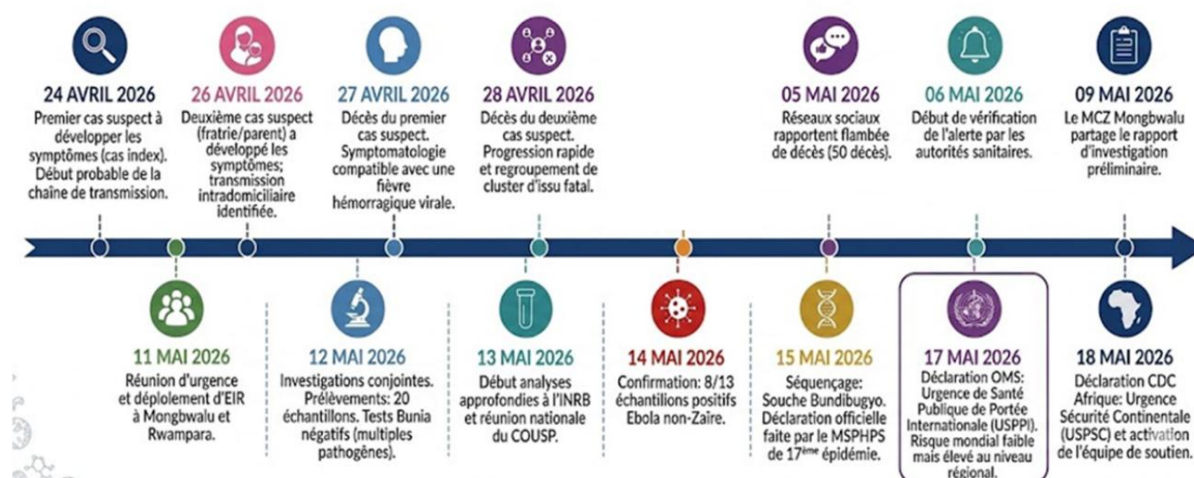


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	2,988	1,238	41.4%	28 / 36 (77.8%)	87
North Kivu*	325	223*	68.6%	11/34 (32.4%)	11
Haut-Uélé	39	21	52.6%	5/13 (38.5%)	0
Tshopo	5	4	80.0%	3/23 (13.0%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,360	1,487	44.3%	48 out of 140	98

* Harmonisation of number of deaths by health zone in the province of North Kivu (+ 11 deaths).

Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 88.9% of confirmed cases (n = 2,988) and 83.3% of deaths (n = 1,238), resulting in a case fatality rate of 41.4%. The health zones of Bunia (801 cases), Rwampara (585), Mongbwalu (528 cases), Nizi (360), Lita (121 cases), and Nyankunde (114) remain the most affected, accounting for approximately 84.0% of reported cases at the provincial level and 74.7% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	2,988	1,238	41.40%	87	16	15	31
Adja	11	1	9.10%	0	0	0	0
Aru	5	5	100.00%	0	0	0	0
Ariwara	7	1	14.30%	0	0	0	0
Aungba	5	4	80.00%	0	0	0	0
Bamboo	57	12	21.10%	0	0	1	1
Boga	1	1	100.00%	0	0	0	0
Bunia	801	232	29.00%	28	5	7	12
Damascus	16	8	50.00%	0	0	0	0
Drodro	10	5	50.00%	0	0	0	0
Fataki	17	6	35.30%	0	0	0	0
Gethy	1	0	0.00%	0	0	0	0
Kambala	2	0	0.00%	0	0	0	0
Kilo	28	11	39.30%	5	0	0	0
Komanda	36	25	69.40%	1	1	0	1
Lita	121	67	55.40%	4	1	0	1
Logo	9	3	33.30%	0	0	0	0
Lolwa	4	1	25.00%	0	0	0	0
Mahagi	4	1	25.00%	1	0	0	0
Mambasa	9	4	44.40%	0	0	0	0
Mandima	16	12	75.00%	3	3	0	3
Mangala	100	64	64.00%	0	0	0	0
Mongbwalu	528	252	47.70%	22	5	3	8
Nia-Nia	92	48	52.20%	2	1	0	1
Nizi	360	172	47.80%	3	0	1	1
Nyankunde	114	31	27.20%	0	0	0	0
Rimba	8	4	50.00%	0	0	0	0
Rwampara	585	244	41.70%	17	0	3	3
Tchomia	41	24	58.50%	1	0	0	0
North Kivu*	325	223*	68.60%	11	5	14	19
Blessed	46	37	80.40%	2	1	2	3
Butembo	75	61	81.30%	2	1	11	12
Goma	1	0	0.00%	0	0	0	0
Kalunguta	5	3	60.00%	0	0	0	0
Katwa	145	91	62.80%	5	3	0	3
Kyondo	12	7	58.30%	0	0	0	0
Mabalako	1	0	0.00%	0	0	0	0
Masereka	4	0	0.00%	0	0	0	0
Musienene	31	20	64.50%	2	0	1	1
Oicha	4	3	75.00%	0	0	0	0
Vuhovi	1	1	100.00%	0	0	0	0
South Kivu	3	1	33.30%	—	—	—	—
Miti-Murhesa	3	1	33.30%	0	0	0	0
Haut-Uélé	39	21	53.80%	—	—	—	—
Boma	7	4	57.10%	0	0	0	0
Mangbetu							
Isiro	12	7	58.30%	0	0	0	0

Pawa	8	6	75.00%	0	0	0	0
Rungu	1	0	0.00%	0	0	0	0
Wamba	11	4	36.40%	0	0	0	0
Tshopo	5	4	80.00%	—	—	—	—
Lubunga	1	0	0.00%	0	0	0	0
Makiso-Kisangani	3	3	100.00%	0	0	0	0
Mangobo	1	1	100.00%	0	0	0	0
Total	3,360	1,487	44.30%	98	21	29	50

* Harmonisation of number of deaths by health zone in the province of North Kivu (+ 11 deaths).

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi, Lita and Nyankunde, with transmission continuing in areas already affected, including the Rungu health zone in Haut-Uélé.

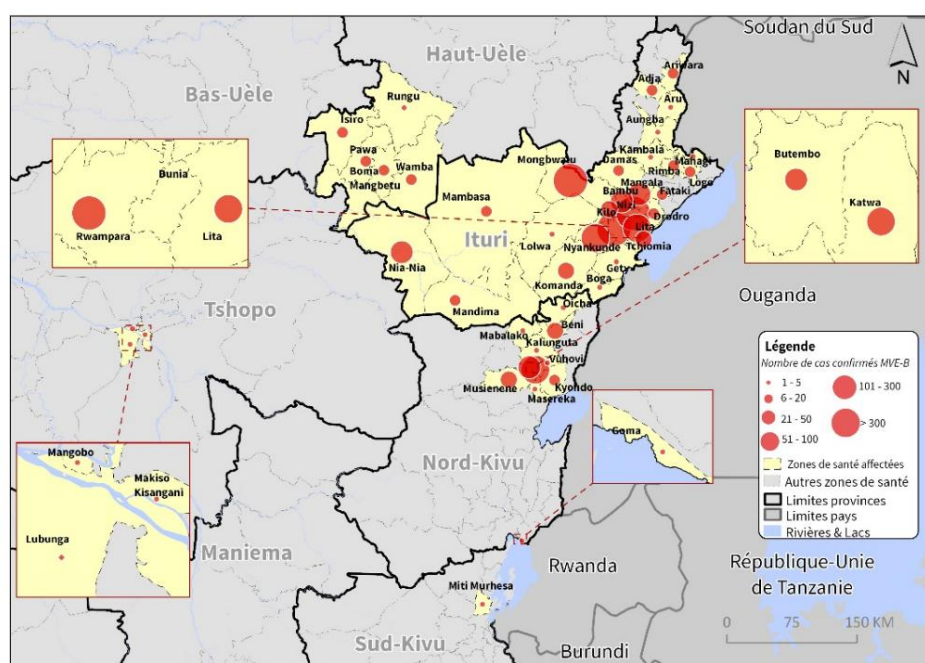


FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

The continued transmission in the Rungu health zone in Haut-Uélé demonstrates the continued transmission in already affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S25 to S30, with fluctuations related to notification delays and the gradual consolidation of data.

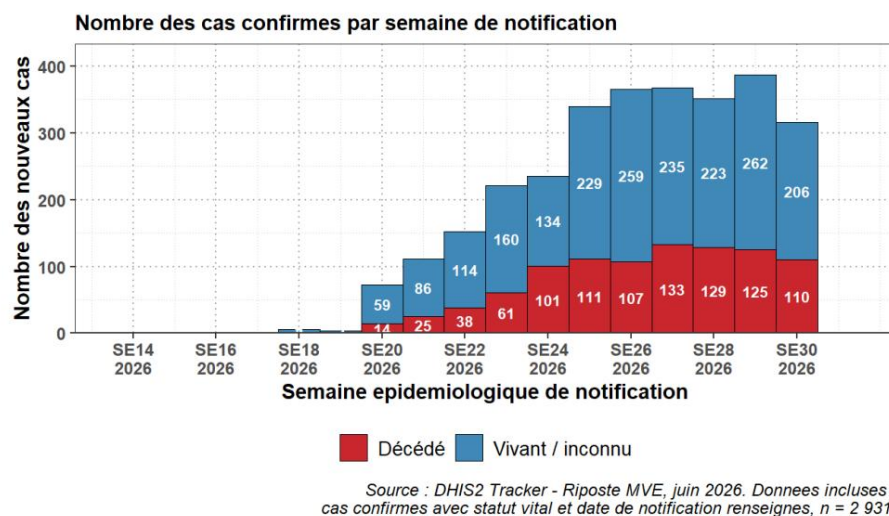


FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

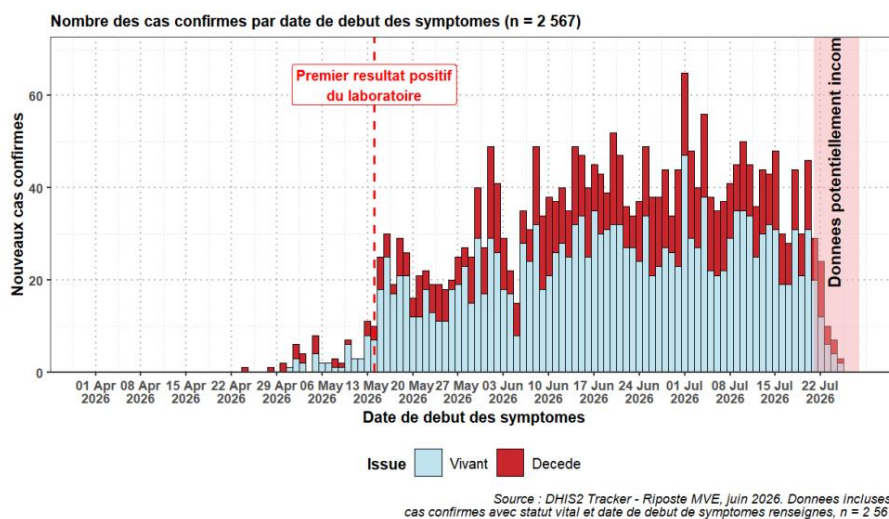


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

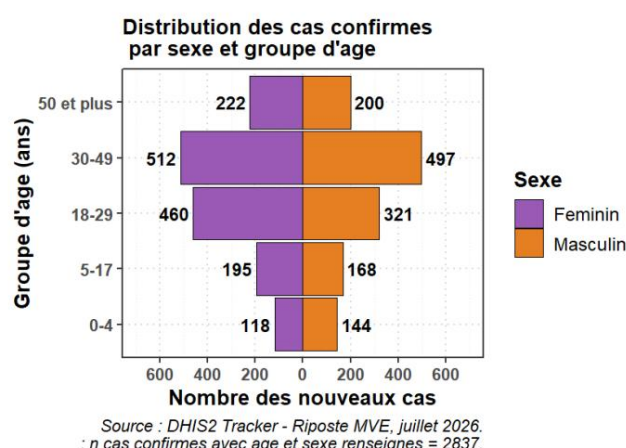
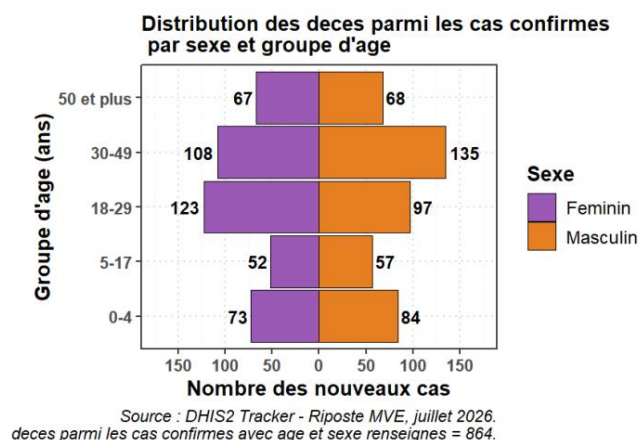


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	17	130	15	0	ND	162
New alerts received	393	858	10	2	ND	1,263
Total alerts for the day	410	988	25	2	ND	1,425
Alerts investigated	273	842	25	2	ND	1,142
Investigation rate (24 h)	66.6%	85.2%	100.0%	100.0%	ND	80.1%
Alerts confirmed — live	134	57	22	0	ND	213
Alerts confirmed — deceased (comm.)	56	48	3	1	ND	108
Total suspected cases for today	190	105	25	1	ND	321

In total, 1,425 alerts (including 1,263 new ones) were issued, with 1,142 investigated within 24 hours (80.1%) and 321 validated (213 living cases and 108 community deaths). This improvement reflects the reduction in investigation postponements and the consolidation of data from the affected provinces. The improved investigation rate is linked to the reduction in investigation postponements and the strengthening of surveillance teams in the affected provinces.

3.2 Laboratory: tests and positivity

ŷ **Ituri** : 206 samples documented in the collection network, of which 87 were positive (71 living and 16 deceased), i.e., a positivity rate apparent of 42.2%.

ŷ **North Kivu** : 100 samples received and analyzed, of which 11 were positive (6 living and 5 dead), representing a positivity rate of 11.0%.

ŷ **Haut-Uélé** : No samples received and analyzed to date.

ÿ **Tshopo** : 3 samples received and analyzed. All of these samples came back negative after analysis.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri*	12,500	9,706	77.6%
North Kivu*	4,908	4056	82.6%
Tshopo	30	30	100.0%
Haut-Uélé	150	90	60.0%
South Kivu (end of monitoring)	0	0	N / A
Total	17,588	13,882	78.9%

*In Ituri and North Kivu, the monitoring rate was calculated only for the health zones that reported. The health zones of Bunia and Mutwanga did not have data available at the time of consolidation of the reports.

Follow-up of contacts remains insufficient at the national level (78.9%), below the operational threshold of 95%. Iturin ensured follow-up of 9,706 contacts out of 12,500 (77.6%). North Kivu achieved a performance of 4,056 contacts seen out of 4,908.

(85.6%). Haut-Uélé'n'asuiquie90 contacts out of 150 (60.0%). La Tshopoma maintains a high performance with a 100.0% follow-up rate. Follow-up is closed in Sud-Kivu.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 2 alerts notified

Two (2) live alerts have been notified and validated: one at PoCMud'zibala, referred to CTedeBunia, and one at PoCDGRPI Nizi, whose transfer to the CTE was refused.

North Kivu — 2 alerts notified

Two (2) alerts notified, one of which was validated, and one lifeless body intercepted.

Tshopo — 1 alert notified

One (1) alert has been notified. No live alerts have been validated nor have any bodies been intercepted.

Haut-Uele — 0 alerts notified

No alerts were notified to the various PoE/PoC devices activated in the province.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (16/16)	100% (9/9)	21% (4/19)	30% 70.2%**	
Number of people who have switched to PoE/PoC	112 101	72,711	11,559	ND	ND	196,371*
% of travelers screened	94%	97.86%	98%	100%	100%	95.4%*
% of travelers who washed their hands	93%	97.86%	98%	98.5%	100%	95.4%*
% of travelers made aware	98.9%	97.86%	100%	100%	100%	98.6%*
Number of alerts notified	2	2	0	1	0	5
Number of validated live alerts	2	1	0	0	0	3
Number of bodies recovered today	0	1	0	0	0	1
Number of problematic contacts intercepted today	1	0	0	0	0	1
Percentage of validated live alerts referred to the CT/CTE	50.0% (1/2)	100% (1/1)	N / A	N / A	N / A	66.7% (2/3)

Indicators	Ituri	North Kivu	South Kivu	Tshopo Haut-Uélé	Overall
% of lifeless bodies swabed	N / A	100% (1/1)	N / A	N / A	100% (1/1)
Number of positive results of referred suspected cases	0	0	0	0	0

In total, five (5) alerts were notified to the PoE/PoC, including three validated live alerts and one intercepted dead body. Two live alerts were referred to CT/CTE (66.7%) and the lifeless body was swabed.

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)

Indicator		Ituri	North Kivu Haut-Uele	Tshopo Together	
Number of beds		808	141	ND	949
Patients in bed (Day -1)		538	181	ND	719
Total admissions (24 hours)		92	48	ND	140
Total admissions		630	229	ND	859
Exits	Deceased	19	3	ND	22
	No case	49	36	ND	85
	Healed	13	1	ND	14
	Escapees	5	0	ND	5
	Transferred to the HGR	0	0	ND	0
Total outflows (24 h)		86	40	ND	126
Patients in isolation (end of day),		544	189	ND	733
including confirmed cases		290	23	ND	313
including suspects		254	166	ND	420
Occupancy rate		67.3%	134.0%	ND	77.3%

In total, 733 patients were in isolation, including 313 confirmed cases and 420 suspected cases, for an overall occupancy rate of 77.3 % (733 patients for 949 beds), with 859 admissions and 126 discharges (14 recovered, 22 deceased, 85 non-cases, and 5 escapees). Ituri accounts for the majority of the burden with 544 patients in isolation (290 confirmed, 254 suspected) out of 808 beds, representing a 67.3% occupancy rate. In North Kivu, 189 patients are in isolation (23 confirmed, 166 suspected) for an occupancy rate of 134.0% (141 beds). Haut-Uele and Tshopo did not provide data at the time of compilation.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	13 (86.7%)	1 (100%)
Confirmed cases are being monitored.	127 (68.3%)	2 (100%)
New suspected cases	32 (47.8%)	12 (100%)
Suspected cases are being monitored	148 (75.1%)	7 (100%)
Laboratory results announced (including positive ones)	41 (18)	4 (3)
Children separated — nursery / community	3/0	0/0
CTE support staff	291	16

SMSPS Key Indicators	Ituri	North Kivu
Frontline Personnel (PPL)	31	18
Members of households and communities	33	0
EDS supported (SMSPS)	2	0

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD

Health Zone	Confirmed cases	Cured. In hospital.	Death	
Bunia	35	18	2	15
Rwampara	33	24	2	7
Mongbwalu	16	6	2	8
Nyankunde	14	11	1	2
Nia-Nia	10	3	7	0
Lita	4	1	2	1
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Logo	1	0	1	0
Total — lethality 30.1%	123	67	19	37

PPL = healthcare workers and frontline providers.

In North Kivu, 11 people are confirmed, among whom 3 have died.

7 Pillar-Based Response Activities

COORDINATION Holding the coordination meeting with the various partners; Monitoring field activities	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ŷ Ituri: Continuation of the needs assessment PCI inputs within the framework of continuity of services and essential care; Analysis of indicators drawn from DHIS2 (ongoing); Development and Budgeting of the CSSE pillar plan.
LOGISTICS ŷ Ituri: 38 vehicles and 11 ambulances (PNC, HGR Bunia and 9 ZS) were made available to the various pillars to ensure the transfer of cases and the mobility of the pillars on the ground). ŷ North Kivu: Handover of 2 ambulances to the Beni Health Zone for Ebola response activities; Delivery of supplies to the INRB laboratory; Data collection and Drafting section reports; Receiving EDS inputs from partner FHI 369; Delivering nutritional support inputs to the health zones of	PCI / WASH ŷ North Kivu: Decontamination of 3/31 households of confirmed cases, including 2 in Katwa and 1 in Butembo Decontamination of 10 ESS (Emergency Situations) of stay of cases, including 2 at Butembo, 6 in Katwa and 2 in Musienene; Incomplete PCI kit allocation in 5 ESS in Butembo; Scorecard evaluation PCI in 1 ESS in Butembo with an average score of 22.5% (CS Vutike); Monitoring and support of 141 social enterprises, including: 20 to Beni, 32 in Butembo, 7 in Katwa, 4 in Kyondo, 2 in Musienene and 76 in Oicha; Briefing of 490 social economy service providers during formative supervisions, including: 60 in Beni, 113 in Butembo,

Beni: (250 cartons) and Mutwanga Health Zone (200); Receipt of the 2nd batch of nutritional support supplies (2411 cartons of Plumpy'Nut, 11 cartons of F100 and 14 cartons of F75 and PCI supplies intended for the Health Zones of Butembo, Katwa and Beni/donation from UGPDSS	35 in Katwa, 23 in Kyondo, and 259 in Oicha; Community deaths that benefited from a DHS (Death Health Survey): 43, including 6 in Beni, 14 in Butembo, 21 in Katwa, 5 in Kyondo, and 2 in Musienene; Community deaths that were the subject of SWAB (Swiss Health Assessment Board): 54, including 11 in Beni, 1 in Butembo, 25 in Katwa, 5 in Kyondo, and 2 in Musienene ÿ Tshopo : Poor waste management, Insufficient receptacles, Non-functional sorting service, Staff not trained in PCI and Insufficient bins.
COMMUNICATION (RCCE / CREC) ÿ Ituri: 2088 people were reached by awareness messages delivered by RECO 54 in households in 1 out of 4 Health Zones (NIA-NIA, NIZI); 256 people were reached by awareness topics during educational talks held by the community actors in health zone (NIA-NIA, NIZI). ÿ North Kivu: Support for the care pillar ; Ceremony for the unloading of a military patient at the CTE-Beni: on this occasion, messages were sent to other military personnel, representatives of the motorcyclists present and the community who came for a guided tour of the CTE. ÿTshopo: 15,868 people were reached during awareness campaigns. (ZS Mangobo, Kabondo, Tshopo, Lubunga and Makioso Kisangani) ; people were reached during door-to-door awareness campaigns at the weather station and Sotraco in the city of Kisangani. ÿHaut-Uélé: Briefing with the CREC teams from the ZS d'Isoiro with a view to intensifying interventions by CREC in the affected AS; visit of 27 Recos to households aimed to convey messages on the importance of EDS to household leaders.	HOLISTIC CARE (HCC) ÿ Ituri: 13 new recoveries; 544 patients in isolation, 92 new admissions and 86 discharges including 19 deaths; 988 people received hot meals; Reception of equipment for biochemistry (High flow and Piccolo + Accessories) at the Rwampara CTE in Ituri. ÿ North Kivu: Unloading of a military medic at the Beni CTE. ÿ Tshopo: No new admissions; 2 non-cases discharged and no patients hospitalized. Experimental treatment continues for 2 contacts (Day 10) at the FEDI Health Center in Mangobo.
RESEARCH, INNOVATION & VACCINATION ÿ Ituri: Identifying gaps, halting the spread of the BDBV virus epidemic: validated protocol and preparation for launching investigations (Africa CDC support); Study of the dynamics of interprovincial and cross-border spread from the epicenter: causes: uncontrolled movement of the sick and deceased, many uncontrolled border crossings, corruption at intra- and interprovincial.	Dignified and secure burials (DSM) ÿ Ituri: Closure of the training of the 2 teams (Niania and pk 51); Day 2 of the training of the 2 EDS teams in the Fataki Health Zone, AS Jina; Continuation of the training of the two teams in Kasenyi; Day 3; Start of the training of the 2 ECUMER teams in the AS MWANGA and KUNDA in the RWAMPARA Health Zone; Reception of 10 cartons of water by the EDS level thanks to the support of FHI 360. ÿ North Kivu: 37 EDS carried out, 39 bodies swabbed and secured out of 42 alerts; escorted during the burial of a confirmed case at the cemetery Paida; a disturbance of public order that occurred during

The burial was brought under control by security forces.

PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH)

ÿ Ituri: 107 Participants (55 Women and 52 Men) bringing the total to 2,597 people including 1,482 Men and 1,115 women already briefed and having signed the commitment to good conduct in 5 health zones of Ituri (Bunia, Rwampara, Mongbwalu, Lita and Aru) and revitalization of the 12 CBCMs in Mongbwalu; 153 Participants (80 F/F and 73 M/G) bringing the total to 15,329 people including 7,995 Men/boys and 7,334 women/girls sensitized in the health zones of Bunia, Rwampara, Aru and Mongbwalu.

8 Main challenges, impacts and actions required

Challenge No.		Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials	Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT: occupancy of 134.0% in North Kivu (bed capacity: 141) and 67.3% in Ituri (544 patients for 808 beds);	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.
4	Low contact tracing performance in Ituri (77.6%); North-Kivu 82.6% and Haut-Uele 60.0	Non-transmission chains interrupted	Train, deploy and motivate community liaison officers (RECOs); strengthen supervision; investigate the observed drop in Ituri
5	Insufficient number of ambulances and pre-positioning at PoC	Delays in transfer and isolation	Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (175/1170, i.e. 15%)	pupil infections Nosocomial risk (123 cases of PPL, including 37 deaths in Ituri and 11 PPL (including 3 deaths in North Kivu)	Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of laboratories proximity (Butembo and Mambasa)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Financial of the resources shortcomings	Limitation of the implementation of pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Unloading ceremony for a military ward at the CTE-Beni in North Kivu

FOR ANY INFORMATION

ADDITIONAL INFORMATION, PLEASE
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